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Case Number: 23SMCV05413 Hearing Date: July 9, 2026 Dept: 205

Superior Court of California

County of Los Angeles

Beverly

Hills Courthouse | Department 205

ASHLEY MATTHAU; and KATHERINE
PARNELLO,

Plaintiffs,

v.

GARTH FISHER, M.D. (fka Donald G.
Fisher); GARTH FISCHER, M.D., a Medical Corporation; and DOES 1 through 20,
inclusive,

Defendants.

Case No.:

23SMCV05413

Hearing Date: 07/09/26

Trial Date:

03/08/2027

[TENTATIVE] RULING RE:

Motion for Summary Judgment or,

In the Alternative, Summary Adjudication

Background

This is a medical malpractice case alleging the following:

Plaintiff Ashley Matthau sought medical care and treatment from Kristi Funk, M.D. in relation to a palpable right breast mass. On that same day, Dr. Funk examined Ms. Matthau and recommended the mass be removed. Dr. Funk understood at that time that Ms. Matthau was also a patient of Garth Fisher, M.D., a plastic surgeon. Dr. Funk and Plaintiff discussed that the mass could be excised at the same time as a procedure by Dr. Fisher. (Stevens Decl. ¶ 8(a).) Dr. Funk emailed Dr. Fisher, acknowledging that Dr. Fisher intended to excise the mass without the assistance or involvement of Dr. Funk. Dr. Funk indicated this plan was “fine.” (Id. ¶ 8(b).)

Plaintiff argues that the change in plan was not Dr. Funk’s choice. Rather, Dr. Fisher’s office unilaterally, and without informing Plaintiff beforehand, told Dr. Funk’s office that she was not needed. This decision was made by Dr. Fisher, without input or agreement by Dr. Funk. (Flagg Decl. ¶ 10(e).) Dr. Fisher used misrepresentations to exclude Dr. Funk, falsely telling her that Ms. Matthau wanted him to excise the tumor, which elicited a vague response “it’s fine,” and falsely telling Ms. Matthau that Dr. Funk agreed with the change. (Plaintiff’s Statement of Facts (“PSOF”) Nos. 53-54.)

Plaintiff confirmed she attended pre-surgical appointments with Dr. Fisher and was made aware of the surgical plan. Prior to the surgical procedure, the plaintiff signed informed consents indicating Dr. Fisher would perform the procedure and indicating which procedures would be performed by Dr. Fisher. The procedures to be performed were a right breast biopsy or lumpectomy and standard scar revision. (Id. ¶ 8(c).) Plaintiff argues that the informed consent was procured through fraud. Plaintiff claims that while she knew Dr. Fisher was performing the procedure rather than Dr. Funk, Dr. Fisher lied about Dr. Funk’s agreement to the change of plans. (PSOF 54.) Dr. Fisher did not disclose that he told Dr. Funk that Plaintiff wanted him to perform the procedure. (Ibid.) Plaintiff also claims that Dr. Fisher lied to her

about where the procedure should be performed. Dr. Fisher's office told Plaintiff that the insurance would not cover the procedure. (Id. ¶ 57.) However, Plaintiff claims that her insurance would have covered the procedure if it had been performed at a hospital, rather than Dr. Fisher's surgical center. (Id. ¶¶ 64, 65.)

On August 30, 2022, Plaintiff underwent scar revision to the right forehead and excision of tumor from the right breast. The procedure was performed at Dr. Fisher's surgical center. (Id. ¶ 8(d).) On September 1, 2022, a pathology report from Cedars-Sinai Medical Center indicates "phyllodes tumor approaches to than .1 cm of inked margin and is focally present at ink." (Id. ¶ 8(e).) On September 19, 2022, Dr. Fisher phoned the pathologist with Plaintiff present. They discussed that there was positive tumor at the margin of the pathology specimen. (Id. ¶ 8(f).)

Plaintiff claims that the first surgical procedure was not successful because Dr. Fisher failed to take out sufficient margins, leaving cancer in Ms. Matthau's body. This was particularly problematic because Dr. Funk had sent Dr. Fisher an email instructing him of the need to take clear margins. Despite admonition from a leading oncologist, Dr. Fisher did not remove a sufficient amount of tissue. Dr. Fisher later admitted his fault and apologized by text and saying he could not see the cancer cells. (PSOF 67, 71.)

On September 20, 2022, Plaintiff underwent right breast wide re-excision with specimen mammography. Findings during the procedure indicate grossly clear margins obtained with no clinically apparent residual tumor. Dr. Funk was the surgeon and Dr. Fisher was the assisting surgeon. (Id. ¶ 8(h).)

On January 16, 2023, Plaintiff returned to Dr. Fisher, complaining of breast rippling to the right breast. Dr. Fisher recommended changing the implants. (Id. ¶ 8(i).) Plaintiff signed multiple informed consents for the planned surgical procedure, which was noted as bilateral breast implant exchange (silicone) and bilateral breast augmentation. (Id. ¶ 8(j).)

Dr. Fisher performed the procedure on January 31, 2023. On a follow up visit, Plaintiff indicated she believed her old breasts looked better than the new breasts. Dr. Fisher told Plaintiff she could go back to her old breasts, but this might result in rippling which caused the plaintiff's

previous concerns. (Id. ¶ 8(l).)

On February 14, 2023, Plaintiff signed multiple informed consents for another planned surgical procedure, a bilateral breast implant exchange (silicone) and bilateral breast augmentation. Specifically, Plaintiff signed a High Risk Consent form detailing the increased risks for the planned procedure. (Id. ¶ 8(m).)

The procedure took place on February 28, 2023. In a follow up visit, Plaintiff said she loved her breasts. Dr. Fisher conducted an exam and instructed the plaintiff to return for further evaluation in one week. Plaintiff did not return to Dr. Fisher and sought care elsewhere. (Id. ¶ 8(o).)

Dr. Fisher's medical expert, Dr. Grant Stevens, is a board-certified plastic surgeon, a professor at the USC School of Medicine, and an expert consultant for the Medical Board of California. He opines that Dr. Fisher acted at all relevant times within the standard of care related to his care and treatment of Ms. Matthau. (Id. ¶ 9.) According to Dr. Stevens, based upon Dr. Fisher's experience as a breast surgeon, it was acceptable for him to perform the tumor excision. He did not require assistance by Dr. Funk. (Id. ¶ 11.) And it was appropriate to perform the procedure at the surgical center, as Dr. Funk also testified. (Ibid.)

As to whether Dr. Fisher properly excised the tumor, Dr. Stevens opines that assuming "the margins were not clear, this is a known risk of the surgical procedure. In a surgical procedure like the one performed by Dr. Fisher on August 30, 2022, there is always the possibility that the margins will not be cleared and residual tumor will remain." (Id. ¶ 13.) "This can and does occur in the absence of negligence. Further, there is always a risk a subsequent procedure will be required to address any residual tumor." (Id. ¶ 14.)

As to the procedure to replace Plaintiff's breast implants, Dr. Stevens opines that the timing of this procedure from the re-excision procedure by Dr. Funk was reasonable and appropriate and allowed Plaintiff time to heal from her prior procedure. According to Dr. Stevens, there is no indication the procedure was performed below the standard of care. (Id. ¶ 19.) Following a procedure like the one performed by Dr. Fisher, Dr. Stevens opines there is always the possibility the patient may not like the aesthetic

appearance of the breasts and additional surgeries may be performed. This can and does in occur in the absence of negligence. (Ibid.)

Dr. Stevens concludes that Dr. Fischer's medical treatment did not fall below the standard of care, and no act or failure to act on the part of Dr. Fisher, to a reasonable degree of medical probability, caused or contributed to the plaintiff's claimed injuries. (Id. ¶ 25.)

In contrast to Dr. Stevens, Plaintiff's medical expert, Dr. Gwenevere Flagg opines that Dr. Fisher's treatment of Ms. Matthau fell below the standard of care. Dr. Flagg opines that Dr. Fisher relied exclusively on his "unaided vision" to determine what tissue to excise and to set margins. Dr. Flagg opines that in doing so Dr. Fisher fell below the standard of care, which requires the use of diagnostic instruments, which would be available at a hospital setting but not necessarily at a surgery center. (Flagg Decl. ¶ 15.)

Dr. Flagg also attests that if the procedure had been performed at the hospital, the pathology report would have indicated that the margins were not clear and that additional tissue had to be removed. Performing the excision procedure in a hospital with immediate pathology available would have made the second surgery unnecessary, would have resulted in less tissue being removed, and would have made the restoration easier and more cosmetically appealing. (Id. ¶ 14(a).)

Dr. Flagg further opines that Dr. Fisher's decision to exclude Dr. Funk, a noted oncologist, from the excision procedure fell below the standard of care as he should have deferred to a specialist like Dr. Funk when performing the procedure at issue. (Id. ¶ 13.) In addition, Dr. Flagg attests that by obtaining Ms. Matthau's consent to the procedure, while omitting material facts, Dr. Fisher's conduct fell below the standard of care. (Id. ¶ 10.)

This hearing is on Defendants' motion for summary judgment or in the alternative for summary adjudication, which was filed on August 8, 2025. Relying on the declaration of their medical expert, Dr. Stevens, Defendants argue that Dr. Fisher's treatment did not fall below the standard of care or cause Plaintiff's injuries and therefore, there is no triable issue on Plaintiff's claim for medical malpractice.

On October 23, 2025, the Court held hearing on Plaintiff's motion

to disqualify Dr. Stevens and granted Plaintiff's request to set an Evidentiary Hearing Re Disqualification of Dr. Stevens.

On November 24, 2025, the Court held the Evidentiary Hearing and denied Plaintiff's motion to disqualify Dr. Stevens as defense expert.

On February 23, 2026, the Court granted the parties' stipulation to continue the date of the hearing. The Court continued the hearing to its current date on July 9, 2026, ordering that Plaintiff may file supplemental opposition no later than June 25, 2026, and Dr. Fisher may file his reply to the supplemental opposition no later than July 2, 2026.

On June 18, 2026, Plaintiff filed her supplemental opposition.

On July 2, 2026, Dr. Fisher filed supplemental reply papers.

On July 7, 2026, Plaintiff filed a document arguing that Dr. Fisher's argument that he was never served with the supplemental opposition is false. Plaintiff attached proof of service.

Motion for Summary Judgment and Summary Adjudication Standard

The function of a motion for summary judgment or adjudication is to allow a determination as to whether an opposing party cannot show evidentiary support for a pleading or claim and to enable an order of summary dismissal without the need for trial. (*Aguilar v. Atlantic Ritchfield Co.* (2001) 25 Cal.4th 826, 843.) Code Civ. Proc. §437c(c) "requires the trial judge to grant summary judgment if all the evidence submitted and 'all inferences reasonably deducible from the evidence' and uncontradicted by other inferences or evidence, show that there is no triable issue as to any material fact and that the moving party is entitled to judgment as a matter of law." (*Adler v. Minor Healthcare Corp.* (1992) 7 Cal.App.4th 1110, 1119.) "The function of the pleadings in a motion for summary judgment is to delimit the scope of the issues; the function of the affidavits or declarations is to disclose whether there is any triable issue of fact within the issues delimited by the pleadings." (*Juge v. County of Sacramento* (1993) 12 Cal.App.4th 59, 67).

As to each claim as framed by the complaint, a defendant moving for summary judgment must satisfy the initial burden of proof by

presenting facts to negate an essential element of the claim(s) or by establishing an affirmative defense. (Code Civ. Proc. § 437c(p)(2); *Scalf v. D.B. Log Homes, Inc.* (2005) 128 Cal.App.4th 1510, 1520.) Courts “liberally construe the evidence in support of the party opposing summary judgment and resolve doubts concerning the evidence in favor of that party.” (*Dore v. Arnold Worldwide, Inc.* (2006) 39 Cal.4th 384, 389.)

Once the defendant has met its burden, the burden shifts to the plaintiff to show that a triable issue of one or more material facts exists as to that cause of action or a defense thereto. (*Id.*) To establish a triable issue of material fact, the party opposing the motion must produce substantial responsive evidence. (*Sangster v. Paetkau* (1998) 68 Cal.App.4th 151, 166.)

In a case involving a cause of action for medical malpractice, “[w]hen a defendant moves for summary judgment and supports his motion with expert declarations that his conduct fell within the community standard of care, he is entitled to summary judgment unless Plaintiff comes forward with conflicting expert evidence.” (*Munro v. Regents of Univ. of Cal.* (1989) 215 Cal.App.3d 977, 985.)

Requests for Judicial Notice

No

requests for judicial notice were filed.

Evidentiary Objections

Plaintiff only filed an objection to Dr. Stevens’s declaration, seeking to disqualify the expert. That objection is overruled.

The Court rules on Defendants’ objections as follows.

* Objection to the Dr. Flagg’s declaration: overruled.

* Objections to Plaintiff’ separate statement: overruled.

* Objection and motion to strike the entire declarations of Plaintiff and Katherine Parnello are overruled and denied, respectively.

Analysis

Defendants move for summary judgment or, in the alternative, summary adjudication of all claims in the operative Second Amended Complaint (SAC) and Plaintiff's request for punitive damages.

First Cause of Action for Medical Malpractice/Negligence

“[I]n any medical malpractice action, the plaintiff must establish: “(1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting from the professional's negligence.” [Citations.]’ [Citation.]” (Hanson v. Grode (1999) 76 Cal.App.4th 601, 606 (Hanson).)

““California courts have incorporated the expert evidence requirement into their standard for summary judgment in medical malpractice cases. When a defendant moves for summary judgment and supports his motion with expert declarations that his conduct fell within the community standard of care, he is entitled to summary judgment unless the plaintiff comes forward with conflicting expert evidence.” [Citations.]’ [Citation.]” (Hanson, supra, 76 Cal.App.4th at pp. 606–607.)

The Court finds that Defendants have met their initial burden of showing that Plaintiff's medical negligence claim lacks merit. As stated above, their expert, Dr. Stevens, testifies that at all relevant times, Dr. Fisher's conduct fell within the standard of care.

However, Plaintiff has also met her burden of showing that triable issues of material fact exist regarding the medical malpractice claim. Her expert testifies that Dr. Fisher's conduct fell below the standard of care.

Therefore, the request for summary judgment and for

summary adjudication of the first

cause of action for medical malpractice/negligence is denied.

Second Cause of Action for Lack of Informed Consent

“Under the informed consent doctrine, “[1] the patient must have the capacity to reason and make judgments, [2] the decision must be made voluntarily and without coercion, and [3] the patient must have a clear understanding of the risks and benefits of the proposed treatment alternatives or nontreatment, along with a full understanding of the nature of the disease and the prognosis.” ‘ [Citation.]’ (Conte v. Girard Orthopaedic Surgeons Medical Group, Inc. (2003) 107 Cal.App.4th 1260, 1267.) The doctrine “imposes a duty on the physician to provide material information about any proposed treatment, such as risks and alternative procedures.” (Ibid.)

Here, the SAC alleges: “When Dr. Fisher obtained Ms. Matthau’s signature(s) on consent forms for the first surgery, that consent was obtained by fraudulently misrepresenting that Ms. Matthau’s oncologist agreed to withdraw from the surgery and to have Dr. Fisher perform the entire procedure. In fact, Dr. Fisher made the decision himself without consulting Ms. Matthau and against the wishes and recommendations of the oncologist.” (SAC, ¶ 44.) “A reasonable person in Ms. Matthau’s position, and Ms. Matthau herself, would have followed the recommendation of the oncologist, who specialized in the removal of cancerous tissue, had Dr. Fisher been truthful about the situation.” (SAC, ¶ 45.) “Based on this fraud and misrepresentation, Ms. Matthau’s execution of consent forms was not based on proper and necessary information and did not constitute the informed consent necessary for Dr. Fisher to perform the procedure to remove cancer tissue.” (SAC, ¶ 46.) “As a direct and legal result of the foregoing, Ms. Matthau’s execution of consent forms was not based on proper and necessary information and did not constitute the informed consent necessary for Dr. Fisher to perform the procedure to remove cancer tissue.” (SAC, ¶ 47.)

The Court finds that Defendants have met their initial burden of showing that Dr. Fisher obtained informed consent that he alone proceeds with the surgery on August 30, 2022. Dr. Fisher testifies as follows. “My long-time patient plaintiff Ashley Matthau was diagnosed with a breast lesion in 2022. Although Surgeon and colleague Dr. Kristi Funk was originally going to exercise the lesion, Ms. Matthau requested that I perform the breast excision, which I ultimately did on August 30, 2022. It was Ms. Matthau’s choice that I perform the breast excision, and this choice was acceptable to both Dr. Funk and me.” (Exh.

3 – Declaration of Garth Fisher, M.D., ¶ 2 [underlining removed]; see also Exh. 2 – Declaration of Kristi Funk, M.D., ¶ 3 [“Prior to the mass excision, my office was notified by Dr. Fisher’s office that Dr. Fisher intended on performing the surgical procedure to excise the right breast mass himself and did not require my assistance”]; id. ¶ 4 [“On July 18, 2022, I prepared an e-mail correspondence to Dr. Fisher wherein I acknowledged that his office informed my office that he intended to excise the mass without my assistance or involvement. In the e-mail correspondence, I indicated this plan was fine”).]

However, Plaintiff has met her burden of showing that triable issues of material fact exist regarding her lack of informed consent claim by testifying that, “[s]hortly before the procedure was to be performed, Dr. Fisher’s office unilaterally, and without informing [Plaintiff] beforehand, told Dr. Funk’s office that she was not needed for the surgery. The initial decision to cut out Dr. Funk was made entirely by Dr. Fisher with no consultation with [Plaintiff or her mother, Katherine Parnello].” (Declaration of Ashley Matthau, filed October 9, 2025, ¶ 16.)

In addition, “[i]nstead of telling [Plaintiff or her] mother that he made a unilateral decision to replace Dr. Funk, Dr. Fisher misrepresented to [them] that Dr. Funk had agreed for him to do the entire procedure without her. [Plaintiff and her mother] learned later that this statement was misleading, and Dr. Funk’s agreement was based on Dr. Fisher falsely telling her that [Plaintiff] wanted Dr. Fisher to repeal Dr. Funk.” (id. ¶ 17.) Plaintiff “was not pleased with [the] change.” (id. ¶ 19.) Indeed, it was Dr. Fisher’s office that had initially and highly recommended Dr. Funk to Plaintiff after she was diagnosed with breast cancer in 2022. (id. ¶ 6.) Plaintiff and her mother were told and ended up confirming that Dr. Funk co-founded the Pink Lotus Breast Center (PLBC), where she excels as an expert in minimally invasive diagnostic and treatment methods for all types of breast disease. (id. ¶ 6.) Therefore, Plaintiff selected Dr. Funk for the surgery “because of [the surgeon’s] expertise in removing breast cancer. [Plaintiff’s and her mother’s understanding] was that she, unlike Dr. Fisher, was experienced and qualified in that surgical procedure and had access to medical equipment designed to enhance the probability of a successful outcome.” (id. ¶ 19.)

In their initial and supplemental reply papers, Defendants argue that there is no triable issue of material fact concerning the informed consent claim because “Plaintiff executed multiple written consent forms identifying the procedures to be performed, the physicians performing those procedures, and the associated risks and benefits.

Dr. Funk's declaration further explains the circumstances surrounding the July 18, 2022 email and confirms that she understood and agreed with the operative plan before surgery." (Suppl. Reply, p. 10:18-22; Initial Reply, p. 7:20-25.)

However, Plaintiff's declaration disputes consenting to Dr. Fisher proceeding with the August 30, 2022 surgery without Dr. Funk. Only the jury can decide whose version of the facts is credible.

Therefore, the request for summary adjudication of the second cause of action for lack of informed consent is denied.

Third Cause of Action for Breach of Contract

The SAC alleges that Dr. Fisher breached the parties' agreement, that Dr. Funk would remove the cancerous tissue and Dr. Fisher would follow to cosmetically restore the breast appearance, by replacing Dr. Funk in the procedure. (SAC, ¶¶ 51, 54.)

In their motion, Defendants do not dispute the existence of the alleged contract. Instead, they argue that breach of contract claim lacks merit for the same reasons discussed above (i.e., Dr. Funk agreed with the plan, and Plaintiff knew before surgery that Dr. Fisher alone will proceed with the surgery on August 30, 2022).

"[E]ven if not modified by an executed oral agreement, 'the parties may, by their words or conduct, waive contractual rights.' [Citations.]" (Park v. NMSI, Inc. (2023) 96 Cal.App.5th 346, 357.) "Waiver is ordinarily a question of fact unless "there are no disputed facts and only one reasonable inference may be drawn.'" [Citation.]" (Id. at p. 358.)

Here, however, the Court has found that triable issues of material fact exist as to whether Plaintiff consented to Dr. Fisher proceeding with the August 30, 2022 surgery alone. Consequently, triable issues also exist as to whether Plaintiff waived the parties' contract terms that Dr. Funk would be involved in the procedure.

Therefore, the request for summary adjudication of the third

cause of action for breach of contract is denied.

Fourth Cause of Action for Medical Battery

The SAC alleges that Dr. Fisher committed medical battery because he performed the surgery to remove cancerous breast tissue from Plaintiff's body after inducing Plaintiff's consent for the surgery by fraudulently misrepresenting that Dr. Funk agreed to withdraw from the surgery. (SAC, ¶¶ 44, 58.)

"[B]attery and lack of informed consent are separate causes of action." (Saxena v. Goffney (2008) 159 Cal.App.4th 316, 324 (Saxena).)

"A claim based on lack of informed consent—which sounds in negligence—arises when the doctor performs a procedure without first adequately disclosing the risks and alternatives." (Saxena, supra, 159 Cal.App.4th at p. 324.)

"In contrast, a battery is an intentional tort that occurs when a doctor performs a procedure without obtaining any consent. (See, e.g., Conte v. Girard Orthopaedic Surgeons Medical Group, Inc. (2003) 107 Cal.App.4th 1260, 1266, 132 Cal.Rptr.2d 855 [a battery 'is an intentional and offensive touching of a person who has not consented to the touching']; Perry v. Shaw (2001) 88 Cal.App.4th 658, 664, 106 Cal.Rptr.2d 70 [doctor committed battery—a claim distinct from professional negligence—when he 'performed an operation to which [plaintiff] did not consent']; Warren v. Schecter (1997) 57 Cal.App.4th 1189, 1194, 67 Cal.Rptr.2d 573 [discussing statute of limitations for 'medical malpractice action arising out of a failure to obtain informed consent to surgery']; Nelson v. Gaunt (1981) 125 Cal.App.3d 623, 634, 178 Cal.Rptr. 167 [battery 'should be reserved for those circumstances where a doctor performs an operation to which the patient has not consented'].)" (Saxena, supra, 159 Cal.App.4th at pp. 324-325.)

Despite those differences, and even if the Court were to find that Defendants have met their initial burden of proving that Plaintiff's medical battery claim lacks merit, Plaintiff has met her burden of showing that triable issues of material fact exist as to the battery claim as well for the following reasons.

"The element of lack of consent to the particular contact is an essential element of battery." (Rains v. Superior Court (1984) 150

Cal.App.3d 933, 938 (Rains) [a case cited by Plaintiff's opposition].)

"In Rains, the plaintiffs, psychiatric patients, agreed to beat each other up after defendants fraudulently represented to them this 'sluggo therapy' would have a therapeutic benefit." (Ashcraft v. King (1991) 228 Cal.App.3d 604, 613.)

The patients sued the psychiatrists for battery and later appealed the trial court's order sustaining the psychiatrists' demurrer without leave to amend. (Rains, supra, 150 Cal.App.3d at p. 935.)

The issue before the Court of Appeal was: "Whether a viable cause for battery has been or might be pleaded turns upon whether plaintiffs' consent to use of physical violence by defendants as a therapeutic treatment may be vitiated by allegations that defendant psychiatrists actually did not administer such violence for any therapeutic purposes but for the improper ulterior purpose of subjugating plaintiffs and controlling their behavior to defendants' personal advantage." (Rains, supra, 150 Cal.App.3d at p. 936.)

The appellate court answered in the affirmative, holding that "deceit as to the therapeutic purpose of contact vitiates consent for the same reason as does deceit as to the lack of professional capacity of the person effecting the purported therapeutic contact." (Rains, supra, 150 Cal.App.3d at p. 940 [italics added].)

"To vitiate consent the mistake "must extend to the essential character of the act itself, which is to say that which makes it harmful or offensive ... rather than to some collateral matter which merely operates as an inducement..." [Citations.]" (Ashcraft v. King, supra, 228 Cal.App.3d at p. 614 [italics added].) "Where ... the patient has expressly conditioned her consent on certain acts being performed by the doctor, the patient has made that condition or event a matter of primary importance. To label such a condition merely 'collateral' is to ignore the patient's 'right, in the exercise of control over his own body to determine whether or not to submit to lawful medical treatment.' [Citation.]" (Ibid.)

Here, the Court finds that triable questions of material fact exist regarding whether (1) Dr. Funk's participation in the surgery on August 30, 2022, was an essential character of the surgery

that Plaintiff consented to, (2) Dr. Fisher used fraudulent misrepresentations to induce Plaintiff's consent to only have him conduct the surgery; and (3) whether such misrepresentations vitiated Plaintiff's consent. In their reply papers, Defendants do not address the above law. In addition, their arguments fail to convince the Court that there are no triable issues of material fact regarding the battery claim.

Therefore, the request for summary adjudication of the fourth cause of action for medical battery is denied.

Fifth Cause of Action for Fraud

The fraud claim alleges the following, among other things. "In or about July 2022, Dr. Fisher made a verbal representation to Ms. Matthau that he and Dr. Funk had decided together that [sic] Dr. Fisher would perform the surgical procedure to remove Ms. Matthau's breast cancer cells, that Dr. Funk would not participate in that surgical procedure, and that the procedure would be performed at Dr. Fisher's surgical center rather than at a hospital." (SAC, ¶ 65.) "At the time Dr. Fisher made these representations, he knew that they were false. The truth was that Dr. Funk was not consulted and did not agree to be excluded from the surgical procedure or for the procedure to be done at any location other than a hospital with the proper and necessary equipment. Dr. Fisher made the decision on his own, to maximize his profits, without regard to the risks, and dictated terms to Dr. Funk." (SAC, ¶ 66.) "At the time Dr. Fisher made these representations, his intent was to induce reliance so that Ms. Matthau would not challenge his decision to exclude Dr. Funk from the surgical procedure and would not challenge his decision to perform the procedure in his surgical center rather than at a hospital." (SAC, ¶ 67.) "Ms. Matthau actually relied on Dr. Fisher's representation and abided by his decision to allow him to perform the procedure without Dr. Funk and in his surgical center rather than at a hospital." (SAC, ¶ 68.)

In their motion, Defendants make the same arguments that Dr. Funk agreed to the surgery, Dr. Fisher believed that Dr. Funk had agreed to the plan, Plaintiff was aware that only Dr. Fisher would be conducting the surgery on August 30, 2022, and it was within the standard of care for Dr. Fisher to perform the surgery at his surgical center instead of a hospital. (Motion, p. 16:8-20.) Defendants further argue that Plaintiff was aware of the insurance issue before the surgical procedure, and Plaintiff's mother was informed before

the surgery that the payment will be out of pocket. (Id. at pp. 16:26-17:7.)

Even if Defendants have met their initial burden of showing that Plaintiff's fraud claim lacks merit, the Court finds that triable issues of material fact exist regarding whether Dr. Fisher obtained Plaintiff's consent through misrepresenting Dr. Funk's lack of participation in August 30, 2022 surgery.

Therefore, the request for summary adjudication of the fifth cause of action for fraud is denied.

Sixth Cause of Action for Breach of Fiduciary Duty

The breach of fiduciary duty claim alleges that "Dr. Fisher breached his fiduciary duties to Ms. Matthau when he took the steps previously alleged to replace Dr. Funk as the surgeon to remove the cancerous tissue and to change the location of the procedure from a hospital to his surgical center." (SAC, ¶ 76.)

Defendants' move for summary adjudication of that claim, making the same arguments as the ones above. (Motion, p. 17:19-24 ["As discussed above, the declaration of Dr. Funk directly refutes the plaintiffs fraud contentions. Further, Dr. Funk and Dr. Stevens agree it was appropriate to perform the procedure at a surgical center. Further Dr. Stevens opined it was appropriate for Dr. Fisher to performed the procedure without Dr. Funk's involvement. The plaintiff knew the surgical plan well before the procedure occurred as the surgical plan was discussed with her and she signed multiple informed consents prior to the same"].)

The Court has found triable issues of material fact exist regarding the allegations underlying the breach of fiduciary duty claim.

Therefore, the request for summary adjudication of the sixth cause of action for breach of fiduciary duty is denied.

Seventh Cause of Action for Invasion of Privacy

The SAC alleges that: "Defendants violated Ms. Matthau's privacy

in at least the following two ways. First, Defendants allowed Ms. Medeiros, who had no medial [sic] basis to be present, to attend sensitive procedures, including surgical procedures. Second, Defendants transmitted images and information to a physician who was not necessary for Ms. Matthau's care and treatment without Ms. Matthau's consent." (SAC, ¶ 83.) "As a direct and proximate result of this invocation of privacy, Ms. Matthau suffered, and continues to suffer, emotional distress, mental discomfort, embarrassment, humiliation, anxiety, and worry." (SAC, ¶ 84.)

Defendants move for summary adjudication of that claim, arguing the following.

"The cause of action for invasion of privacy, as asserted by the plaintiff, is likely governed by either CACTI 1800 or CACTI 1801.

"As for CACTI 1800 (intrusion into private affairs), the plaintiff must show: 1. She had a reasonable expectation of privacy in the pre-operative and operative setting; 2. That Dr. Fisher intentionally intruded in the pre-operative or operative setting; 3. that Dr. Fisher's intrusion would be highly offensive to a reasonable person; 4. The plaintiff was harmed; and 5. Dr. Fisher's conduct was a substantial factor in causing the harm.

"As for CACTI 1801 (public disclosure of private facts), the plaintiff must show: 1. Dr. Fisher publicized private information concerning the plaintiff; 2. A reasonable person in the plaintiff's position would consider the publicity highly offensive; 3. Dr. Fisher knew, or acted with reckless disregard of the fact, that a reasonable person in the plaintiff's position would consider the publicity highly offensive; 4. the private information was not of legitimate public concern; 5. The plaintiff was harmed; and 6. The conduct was a substantial factor in causing the plaintiff harm."

(Motion, p. 18:7-19.)

Defendants argue that the allegation that Dr. Fisher transmitted Plaintiff's image to another physician does not constitute invasion of privacy under the public disclosure theory because (1) the image was transmitted to

only one individual, and (2) Plaintiff signed a photographic video release and consent form. (Motion, p. 19:3-12.)

As for Plaintiff's claim that Ms. Medeiros was allowed to attend sensitive procedures, Plaintiff's mother testified that both she and Plaintiff knew of Ms. Medeiros involvement and presence in the surgical procedures. (Motion, p. 19:13-15.)

The Court finds triable issues of material fact exist as to the invasion of privacy claim at least with regard to Plaintiff's allegation of Ms. Medeiros. As Plaintiff argues, "[k]nowledge does not mean consent [of Medeiros's presence]." (Opposition, p. 15:16; see also Matthau Decl., 24, 25, 26 [testifying that Medeiros's presence during preparation of the procedure and during the surgery made Plaintiff uncomfortable as it reminded her of traumatic, sexual assault events that were similar to the ones Medeiros was being accused of being associated with at the time].)

Therefore, the request for summary adjudication of the seventh cause of action for invasion of privacy is denied.

Eighth Cause of Action for Negligent Infliction of Emotional Distress

"The law of negligent infliction of emotional distress ["NIED"] in California is typically analyzed, as it was in this case, by reference to two 'theories' of recovery: the 'bystander' theory and the 'direct victim' theory." (Burgess v. Superior Court (1992) 2 Cal.4th 1064, 1071 (Burgess).)

"The distinction between the 'bystander' and 'direct victim' cases is found in the source of the duty owed by the defendant to the plaintiff." (Burgess, supra, 2 Cal.4th at p. 1072.)

"[B]ystander liability is premised upon a defendant's violation of a duty not to negligently cause emotional distress to people who observe conduct which causes harm to another." (Burgess, supra, 2 Cal.4th at p. 1073 [emphasis added].)

The California Supreme Court has held that a plaintiff may recover NIED damages based on a bystander theory "only if ... said plaintiff: (1) is closely related to the injury victim; (2) is present at the scene of the

injury-producing event at the time it occurs and is then aware that it is causing injury to the victim; and (3) as a result suffers serious emotional distress—a reaction beyond that which would be anticipated in a disinterested witness and which is not an abnormal response to the circumstances.’ [Citation.]” (Bird v. Saenz (2002) 28 Cal.4th 910, 916–917, citing Thing v. La Chusa (1989) 48 Cal.3d 644, 667–668 (Thing) [italics added].) “Thing’s requirement that the plaintiff be contemporaneously aware of the injury-producing event has not been interpreted as requiring visual perception of an impact on the victim. [However,] [a] plaintiff may recover based on an event perceived by other senses so long as the event is contemporaneously understood as causing injury to a close relative. ([See e.g.] Wilks v. Hom (1992) 2 Cal.App.4th 1264, 1272–1273, 3 Cal.Rptr.2d 803 [plaintiff was in the living room speaking to her children in their bedrooms when she saw, heard and felt one bedroom explode from a gas leak].)” (Bird v. Saenz, supra, 28 Cal.4th at pp. 916–917.)

Direct victim cases, on the other hand, involve “cases where a duty arising from a preexisting relationship is negligently breached.” (Burgess, supra, 2 Cal.4th at p. 1074 [italics added].) For example, in Burgess, a mother sued an obstetrician, alleging that she suffered emotional distress as a result of the obstetrician’s negligence in delivering her baby. (Id. at p. 1070.) The California Supreme Court held that the mother was a direct victim (instead of a bystander) because the obstetrician had a physician-patient relationship with the mother and her fetus. (Id. at p. 1076 [“It is in light of both these physical and emotional realities that the obstetrician and the pregnant woman enter into a physician-patient relationship. It cannot be gainsaid that both parties understand that the physician owes a duty to the pregnant woman with respect to the medical treatment provided to her fetus. Any negligence during delivery which causes injury to the fetus and resultant emotional anguish to the mother, therefore, breaches a duty owed directly to the mother”]; see also id. at p. 1078 [“Under the facts of this case, Burgess is not a ‘bystander’ for purposes of bringing a claim for compensation for damages for her serious emotional distress”].)

Here, the NIED claim is brought by Plaintiff’s mother, Parnello, against Defendants and alleges the following. “By virtue of Dr. Fisher’s knowledge of the family relationship between Ms. Matthau and Ms. Parnello, Defendants’ duty of care extended to include Ms. Parnello.” (SAC, ¶ 88.) “As described [in the pleading], Defendants were [sic] negligent on numerous occasions while providing medical services to Ms. Matthau

and while Ms. Parnello was present to observe and have personal knowledge of the negligence and severe physical and emotional injuries Ms. Matthau suffered as a result of that negligence." (SAC, ¶ 89.) "Ms. Parnello suffered, and continues to suffer, emotional distress, suffering, anguish, fright, horror, nervousness, grief, anxiety, worry, shock, humiliation, mental discomfort, embarrassment, and worry. She has suffered damages in an amount not yet precisely attainable, but which she will establish at trial." (SAC, ¶ 90.)

"Defendants' negligence was a substantial factor in causing Ms. Parnello's serious emotional distress." (SAC, ¶ 91.)

Defendants

argue, and the Court agrees, that Parnello's NIED claim is based on a bystander theory. Plaintiff's argument that Parnello was a direct victim is unpersuasive. (See Opposition, pp. 16:13-17:15 [arguing that Parnello was a direct victim because she feared that cancer would remain in her daughter's body, had advanced money to pay for the surgery, and was present at the appointments. However, unlike cases where a pregnant mother sued her obstetrician for the negligent delivery of her baby, there was no such pre-existing relationship between Dr. Fisher and Parnello here].) The Court declines to extend the direct victim theory in a case such as this one without any law directing it to do so.

The Court

finds that Defendants have met their initial burden of proving that Parnello's NIED claim lacks merit. As stated above, to prevail on a bystander theory, an NIED plaintiff must prove that she contemporaneously perceived and understood the event that caused injury to their close relative. Here, Parnello does not dispute that during her deposition, she testified that she did not witness the surgeries and did not witness any of the actual procedures. (Motion, p. 20:20-24.)

The Court

also finds that Parnello has failed to meet her burden of showing that triable issues of material fact exist as to her bystander claim. Parnello argues: "[B]ystander liability is based on the negligence in Dr. Fisher's delay in paying attention to the pathology report. [Citation.] It was negligent to fail to perform the procedure in a hospital where pathology reports would be contemporaneous. Ms. Parnello was present to witness the shock of learning Dr. Fisher waited approximately 3 weeks to learn that he did not leave clear margins and the scurry to schedule an emergency second excision procedure. She was present to see Dr. Fisher's expression of horror when he finally read the pathology report. Her being on the scene to witness that negligence satisfies the requirements for a bystander action."

(Opposition, pp. 20:25-21:3.) However, it was not Dr. Fisher's so-called expression of horror when he read the pathology report that injured Plaintiff. The SAC alleges that Plaintiff was injured by the medical practice of Dr. Fisher, including his failure to perform the surgery with the required standard of care. Therefore, Parnello has failed to meet her burden.

Accordingly,
the request for summary adjudication of the eighth cause of action for NIED is granted.

Punitive Damages

Defendants do not present substantive arguments regarding Plaintiff's request for punitive damages. They only argue that "if the Court grants summary adjudication as to causes of action that could rise to punitive damages [e.g., fraud], then summary adjudication must be granted as to plaintiffs' claims for punitive damages." (Motion, p. 21:14-16.)

The Court has not granted summary adjudication as to causes of action that could give rise to punitive damages.

Accordingly,
the request for summary adjudication of Plaintiff's request for punitive damages is denied.

Conclusion

Based on the
foregoing, the Motion for
Summary Judgment or, in the Alternative, Summary Adjudication is GRANTED IN PART and DENIED IN PART as follows.

The request for
summary adjudication as to the eighth cause of action for negligent infliction of emotional distress is GRANTED.

The motion is
otherwise DENIED.

Dated: July 9, 2026

Edward B. Moreton, Jr.

Judge of the Superior Court